



EVERGREEN COMMUNITY CARE INC.

CULTURAL SAFETY & GENDER-AFFIRMING CARE MANUAL

Cultural Safety and Humility · Indigenous Cultural Safety · Anti-Racism · Language and Communication Access · Religious and Spiritual Practice · Gender-Affirming Care in Everyday Program Life · Consent and Access to Gender-Affirming Health Care — Children and Youth (MCFD) and Adults (CLBC)

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Review Date	July 1, 2027 (annual); also upon any funder or licensing directive, any concern raised by a person in care, family, or community, or BC legislative or regulatory change
Applies To	All Evergreen Community Care residential care programs and 1–2 bed homes serving: children and youth in MCFD care; adults receiving CLBC-funded residential services. All Youth Care Workers / Community Support Workers, Shift Leads, Program Coordinators, Program Managers, and the Executive Director. Volunteers, students, and contractors in the home also work to this manual.
Funder Streams	M = MCFD (children and youth) · C = CLBC (adults) · Both = both streams. Where a requirement is stream-specific it is marked (M) or (C).
Legislative Authority	BC Declaration on the Rights of Indigenous Peoples Act (DRIPA) and UN Declaration on the Rights of Indigenous Peoples (UNDRIP); Child, Family and Community Service Act (CFCSA) — Indigenous provisions and s.14; An Act respecting First Nations, Inuit and Métis children, youth and families (federal); Human Rights Code (BC) — protected

	grounds including Indigenous identity, race, ancestry, religion, gender identity and gender expression, sexual orientation; Infants Act s.17 (mature minor consent); Health Care (Consent) and Care Facility (Admission) Act (HCCCFAA); Personal Information Protection Act (PIPA) and FOIPPA
Funder Standards	MCFD Standards for Staffed Children's Residential Services (SHSS): Standard G.3.2(b) — gender-affirming care training required before duties; Indigenous cultural safety expectations across the standards; children's cultural, spiritual, and identity rights (Standard C and E areas). CLBC standards and contract terms: cultural responsiveness, Indigenous engagement, and non-discrimination provisions per the executed service contract.
Related Documents	Behaviour Support Manual (ECC-BSP-2026-001) — no coercion, dignity-first; Health Care & Consent Policy Manual (ECC-HC-2026-001) — Infants Act and HCCCFAA consent frameworks; Medication Administration Manual (ECC-MED-2026-001); Illness, Injury & Medical Emergency Manual (ECC-IIME-2026-001); Nutrition, Illness & Infection Prevention Manual (ECC-NIP-2026-001) — cultural and religious diets §7; HR Policy Binder (ECC-HR-POL-2026-001) — recruitment, harassment, respectful workplace; 1-2 Bed Home Opening & Compliance Binder (ECC-12BED-2026-001)

 This manual sets one agency standard: **every person in an Evergreen Community Care home is treated with cultural safety, and every person is supported in their gender identity and expression as they define it — every day, from the first shift.** Cultural safety and gender-affirming care are not optional add-ons or the specialty of individual staff. They are basic conditions of care. When a person tells us who they are — their name, their pronouns, their identity, their community, their beliefs — we listen, we act on it, and we do not require them to explain themselves. Failing this standard is a safety issue, not merely a preference issue: the evidence is clear that culturally unsafe and non-affirming care causes measurable harm and that safe, affirming care prevents it.

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VERSION HISTORY

PART 1 — Policy Foundation

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

Residential care can heal or harm. It heals when the people who live there are seen as they are — as members of their families, communities, cultures, and identities — and are supported to live that way, every day. It harms when they are not. This manual makes cultural safety and gender-affirming care the standard of care at Evergreen Community Care, not the personal project of individual staff. Every staff member is trained to this manual; every program operates to it; every person in care can expect it.

Principle	What It Means in Practice
The person is the authority on who they are	A person's identity — their culture, community, language, religion, name, pronouns, gender — is theirs to name. Staff receive it, record it, and act on it. Staff do not require justification, do not interrogate, and do not second-guess. If a person tells us they are Nuuchah-nulth, or Sikh, or lesbian, or a trans woman, that is the answer.
Culture and identity are safety issues	Culturally unsafe or non-affirming care causes real harm — increased distress, disengagement from services, self-harm and suicide risk, and loss of connection. Culturally safe, affirming care measurably improves outcomes. We treat these the same way we treat medication safety or infection control: as core practice.
Indigenous cultural safety is a baseline, not an add-on	BC has adopted DRIPA and reformed law and policy to reflect Indigenous rights and the reality of ongoing colonial harm in health and child welfare (In Plain Sight, 2020; the Truth and Reconciliation Commission Calls to Action). At Evergreen Community Care, Indigenous cultural safety is a baseline standard for every person — Indigenous or not — and specifically shapes how we support Indigenous children, youth, and adults in our homes.
Anti-racism is the work of the whole agency	Racism — including everyday microaggressions, staff comments, roommate incidents, and community responses — is named and addressed. Silence is not neutrality; it protects harm. The PM and ED are responsible for a program where addressing racism is safe and expected, not risky.
Gender-affirming care is a daily practice, not a checkbox	Using a person's name and pronouns, honouring their clothing and grooming choices, protecting their privacy, and supporting access to gender-affirming health care are ordinary duties on every shift. They are provided without staff commentary and without conditions.

Principle	What It Means in Practice
Consent frameworks apply — the person leads	For children and youth (M): the Infants Act supports mature minors consenting to their own health care, including gender-affirming health care where the provider is satisfied of capability (ECC-HC §12). For adults (C): the adult consents for themselves — the presumption of capability governs (ECC-HC Part 4). This manual operates inside those frameworks, never around them.
Humility over expertise	Staff will not know everything about every culture, community, or identity — and are not asked to. Staff are asked to bring humility, to ask respectfully where invited, to learn continuously, and to recognize that expertise about a person's identity belongs to the person and their community.
No coercion, no conditions, no 'later'	Cultural and gender-affirming supports are never rationed, delayed, or used as behaviour management leverage. 'You can wear that when you show us you can behave' or 'we'll use your name when your family is on board' are prohibited practices.

2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Stream	Relevance and Key Requirements
BC Declaration on the Rights of Indigenous Peoples Act (DRIPA), 2019	Both	Aligns BC law with the UN Declaration on the Rights of Indigenous Peoples. Requires the province to work in cooperation with Indigenous peoples and shapes all provincial standards, including health and child welfare, in which Evergreen Community Care operates.
UN Declaration on the Rights of Indigenous Peoples (UNDRIP)	Both	Recognizes Indigenous peoples' rights to self-determination, culture, language, and family; requires free, prior, and informed consent on matters affecting Indigenous peoples. Programmatic implication: Indigenous children, youth, and adults in care remain connected to their communities and cultures as a right, not a privilege.
Child, Family and Community Service Act (CFCSA) — Indigenous provisions	M	The Act and subsequent reforms centre Indigenous children's connection to their families, communities, and Nations. Best

Authority	Stream	Relevance and Key Requirements
		interests include cultural, racial, linguistic, and religious heritage; Indigenous governing bodies have expanded roles in decisions about their children. Programs supporting Indigenous children in care operate consistent with these provisions and the direction of the child's community wherever it is provided.
An Act respecting First Nations, Inuit and Métis children, youth and families (federal, 2019)	M	Affirms Indigenous jurisdiction over child and family services and sets national principles: best interests of the Indigenous child, cultural continuity, and substantive equality. Programs comply with any Indigenous law brought into force in respect of a child in the home.
Human Rights Code (BC)	Both	Prohibits discrimination in services on grounds including Indigenous identity, race, ancestry, place of origin, colour, religion, sex, sexual orientation, gender identity, gender expression, family status, physical or mental disability, and others. Evergreen Community Care is a service provider under the Code.
Infants Act, s.17	M	A minor of any age may consent to their own health care — including gender-affirming health care — where the treating health care provider is satisfied of capability and best interests. ECC-HC §12 governs the program's role.
Health Care (Consent) and Care Facility (Admission) Act (HCCCFAA)	C	Adults are presumed capable. Adults consent to their own gender-affirming and culturally specific health care. Substitute frameworks apply only where a provider determines incapability for a specific decision (ECC-HC Part 4).
MCFD SHSS Standard G.3.2(b)	M	Contracted residential care providers must provide gender-affirming care training to staff before duties. Evergreen Community Care satisfies this through orientation and annual

Authority	Stream	Relevance and Key Requirements
		refreshers on this manual, aligned with Trans Care BC and other authoritative BC clinical guidance.
CLBC contract and standards	C	Require cultural responsiveness and non-discrimination in service delivery for adults with developmental disabilities, including recognition of Indigenous identity and 2SLGBTQIA+ adults. Programs implement per the executed contract.
FOIPPA / PIPA	Both	A person's cultural identity, Indigenous status, immigration status, religion, sexual orientation, and gender identity are among the most sensitive categories of personal information. Collection, use, and disclosure are strictly limited to what is necessary for care and to what the person authorizes.

3. ROLES AND RESPONSIBILITIES

Role	Responsibilities Under This Manual
Youth Care Worker / Community Support Worker	Use each person's name and pronouns from day one and every day thereafter; respect and support each person's cultural, religious, spiritual, and identity practices; complete the trainings required by this manual before unsupervised duties (Section 25); ask respectfully where invited; document the person's culture, identity, and preferences accurately in Appendices A–C as directed by the PC and by the person; interrupt and report any incident of racism, transphobia, homophobia, ableism, or other identity-based harm — including from staff, roommates, family visitors, community, or contractors — same shift; never require a person to educate them as a condition of care.
Shift Lead	All worker responsibilities plus: verify at shift start that Appendix A / B / C plans and their identity and communication information are known to the on-shift team; support the person's participation in cultural, spiritual, and community activities scheduled for the shift; lead the on-shift response to any incident of identity-based harm (Section 23) and ensure the notification chain starts before end of shift.

Role	Responsibilities Under This Manual
Program Coordinator (PC)	Complete the Individual Cultural Care Plan (Appendix A) and the Gender-Affirming Care Plan (Appendix C) with every person at admission and update at every change and at least annually; complete the Indigenous Cultural Plan Supplement (Appendix B) with every Indigenous person in care, in relationship with the person and — as the person authorizes — their family, community, or delegated Indigenous authority; arrange language and interpretation access (Section 8); coordinate cultural, spiritual, and community connections; hold and update the roster of local resources (Indigenous organizations, 2SLGBTQIA+ youth and adult supports, faith communities, interpreter services); receive complaints (Section 23) and support the person through the process.
Program Manager (PM)	Approve every Appendix A/B/C plan; ensure staff training currency (Section 25); resolve escalated concerns and complaints; make funder notifications for identity-based harm incidents in parallel with ECC-IIME §17; sustain program-level relationships with Indigenous organizations and community partners; report quarterly to the ED on QA indicators (Section 24) and any patterns.
Executive Director (ED)	Approve this manual annually; ensure resourcing for training, interpretation, cultural supplies, and community relationships; respond to systemic issues; sign off on Indigenous engagement direction with any Indigenous governing body relevant to a child, youth, or adult in care; represent the agency to funders on cultural safety and gender-affirming care commitments.
Volunteers, students, contractors	Read and sign Appendix F before any contact with people in care; work to this manual for the duration of their involvement; the PC supervises.
Persons in care	Are the authority on their own identity and culture. Participate to the extent they wish in the plans in this manual, name their own supports and communities, and can decline any part at any time without consequence.

4. KEY DEFINITIONS

Term	Definition as Used in This Manual
Cultural safety	An environment where a person feels safe to be who they are — where their identity, culture, and history are respected, where the power imbalance between service provider and service recipient is recognized,

Term	Definition as Used in This Manual
	and where the person judges whether the care they received was safe. Cultural safety is defined by the person, not by the provider.
Cultural humility	A lifelong practice of self-reflection and learning about power, privilege, and difference; approaching people with openness and without assumption; recognizing what one does not know.
Indigenous cultural safety (ICS)	Cultural safety specifically as it applies to Indigenous peoples in colonial systems (including residential care and health care), grounded in acknowledgement of ongoing colonial harm and commitment to reversing it in day-to-day practice. Reflects the direction of In Plain Sight (2020) and the BC ICS training required for staff (Section 25).
Gender identity	A person's own internal sense of their gender. Not visible, not chosen by others, not determined by anatomy or appearance.
Gender expression	How a person presents their gender publicly — clothing, hair, name, voice, mannerisms — which may or may not match others' expectations.
Gender-affirming care	Care and support that affirms the person's gender identity and expression. Ranges from ordinary daily practice (name, pronouns, clothing, safe spaces) to specific health care (which is provided by health care providers under the consent frameworks in ECC-HC).
2SLGBTQIA+	Two-Spirit, lesbian, gay, bisexual, transgender, queer, questioning, intersex, asexual, and other identities. Two-Spirit is an Indigenous identity encompassing a distinct set of understandings of gender, sexuality, and role within many Indigenous cultures; the term belongs to Indigenous peoples and is used only by those who identify with it.
Cisgender	Describing a person whose gender identity is the same as the one assigned to them at birth.
Transgender / trans	Describing a person whose gender identity differs from the one assigned to them at birth. Includes trans women, trans men, non-binary people, and others.
Non-binary	Describing gender identities that are not exclusively 'woman' or 'man'; includes agender, genderfluid, and many other identities.
Outing	Disclosing a person's gender identity, sexual orientation, HIV status, immigration status, or other private information without their permission. Prohibited practice — Section 22.

Term	Definition as Used in This Manual
Deadname	The name a trans or non-binary person was given at birth or previously used, which they no longer use. Deadnaming — using that name — is a harm and is prohibited practice except where legally required and the person is informed.

PART 2 — Cultural Safety

5. CULTURAL SAFETY AND HUMILITY — THE PROGRAM STANDARD

Standard	Program Practice
Assume you don't know	Staff never assume a person's culture, community, religion, or beliefs from their appearance, name, or origin. Culture is not visible; it is what the person tells us. Where nothing has been told or recorded, the answer is that we do not know — not an assumption filled in from stereotype.
Ask respectfully; do not require answers	Where information about a person's culture, faith, or identity would help us support them (food, holidays, funeral practices, gendered care preferences), staff ask openly, once, and accept whatever answer is given — including 'I don't want to say' or 'I'll tell you later'. A person is never required to justify or explain their culture.
Learn the person before the category	A general awareness of a community's traditions is not a substitute for knowing this person. Two people from the same Nation, same faith, or same identity can hold different beliefs and practices. The Cultural Care Plan (Appendix A) is built with each person about themselves — not from a template about their group.
Everyday practice, not exceptional events	Cultural safety is expressed in daily choices — who is on shift when a family visitor comes, whose language is spoken over dinner, whose holiday is on the fridge calendar — not only in ceremonies or celebrations. Small everyday details are what tell the person whether they are safe.
The person judges safety, not the provider	Whether care was culturally safe is decided by the person receiving it. When a person tells us it did not feel safe, that is the fact; the response is inquiry and change (Section 23), not defence.

6. INDIGENOUS CULTURAL SAFETY

Evergreen Community Care operates on the unceded and treaty territories of Indigenous peoples across British Columbia. Indigenous children, youth, and adults are overrepresented in residential care as a direct legacy of colonial systems — Indian Residential Schools, the Sixties Scoop, and continuing removal and

institutionalization. Indigenous Cultural Safety is not a separate initiative; it is a baseline the whole program is built on, and it specifically shapes how we support Indigenous people who live in our homes.

Practice	Standard
Territory acknowledgement — meaningful, not ceremonial	Programs know and acknowledge the Nations on whose territories they operate. Territory acknowledgement is a beginning, not a substitute for action; the actions are in the rest of this Part and in Part 3.
Recognize and address anti-Indigenous racism	Anti-Indigenous racism is documented and pervasive in BC health and social systems (In Plain Sight, 2020). Staff are trained to recognize it in institutional practices (dismissive triage, assumptions about parenting, dismissal of pain reports) and in the program itself. Any staff conduct that reflects it is addressed under HR policy; any external encounter with it — at hospitals, schools, appointments — is documented and reported to the PC/PM.
Trauma-informed and colonial-informed practice	Many Indigenous people in care are living with intergenerational trauma linked to colonial policy. Staff practise trauma-informed care (predictability, choice, collaboration, safety) with awareness that some triggers — institutional settings, uniformed authority, medical settings — carry specific historical weight.
Language and land	Where a person speaks or wishes to reclaim their language, the program supports access (recordings, community classes, language teachers as they are available). Access to land — the person's own territory where possible, or land near the home — is supported as a normal part of care, not an occasional outing.
Relationships, not transactions	The program builds and maintains relationships with local Indigenous organizations, Friendship Centres, and the Nations whose members live in our homes — before we need them. The PM sustains these relationships as core work.
Follow direction from the community	Where a child, youth, or adult in the home is a member or citizen of a Nation or Indigenous governing body, and that body has communicated direction about their care (cultural, ceremonial, family), the program follows that direction as authoritative unless it directly conflicts with another legal requirement, in which case the PM engages the ED and the funder to resolve.

7. ANTI-RACISM AND ANTI-DISCRIMINATION

Category	Standard
Racism, including microaggressions	Racism is prohibited in all forms: overt slurs; jokes;

Category	Standard
	comments about hair, name, food, English fluency, or family; questions about origin or citizenship; assumptions about ability, criminality, or parenting; and institutional practices with racialized impact. This applies to staff, roommates, visitors, contractors, and community actors.
When it happens — the response	Any staff member who witnesses or receives a report of racism (or any identity-based harm) responds the same shift: (1) support and check in with the person harmed; (2) name and interrupt the conduct where safe; (3) notify the PC and complete Appendix E before end of shift; (4) the PC decides on further steps with the PM. Silence is not a response — silence is a decision that harm was acceptable.
Racialized power in the program	The PM ensures that staff assignments, promotions, and complaint response do not fall along racialized lines — including racialized staff carrying disproportionate labour on cultural questions, or racialized people in care being asked to explain or educate. The HR Policy Binder governs; this manual reinforces.
Beyond race — the whole Human Rights Code	The same standards and responses apply to discrimination on any protected ground: religion, disability, sexual orientation, gender identity and expression, family status, age, source of income. This manual centres cultural safety and gender-affirming care because those are its subject; the underlying commitment is to non-discrimination across all grounds.

8. LANGUAGE, INTERPRETATION AND COMMUNICATION ACCESS

- The language(s) each person uses, understands, and wishes to be addressed in are recorded on Appendix A at admission. Language preferences may be different for daily life, health care, and confidential conversations — all three are recorded.
- Where a person's preferred language is not shared by staff, the PC arranges professional interpretation for any significant conversation — care planning, health appointments, complaints, family meetings — through a qualified interpretation service (BC Language Services and health authority services where applicable). Interpretation events are logged on Appendix D.
- Family members, roommates, and other people in care are not used as interpreters for significant conversations. Children (M) are never used as interpreters for adults or for other children. Where a person themselves requests a family interpreter for informal matters, that preference is recorded and respected, with the option always kept open to bring in a professional.
- Where a person uses an augmentative or alternative communication (AAC) system, sign language, or has other communication needs identified in the Health Care Profile (ECC-HC App. A), those supports are provided at every interaction — not only clinical ones. Communication accessibility is cultural safety.

- The program keeps some materials in languages spoken in the home; asks the person what would help; and never treats a language other than English as an inconvenience or a problem to be solved by requiring the person to change.

9. RELIGIOUS AND SPIRITUAL PRACTICE

Requirement	Standard
Freedom to practise	Every person in care is free to hold, practise, and change their religion or spiritual beliefs — or to hold none. The program does not proselytize, direct religious practice, or attach any incentive or consequence to a person's beliefs.
Everyday practice supported	Daily practices — prayer times, dietary observance (with ECC-NIP §7), dress, grooming, ceremonial items, altars or sacred spaces in the person's room, days of rest — are supported as ordinary parts of the day; staff know each person's practices from Appendix A and plan around them, not against them.
Access to community and clergy	Where a person wishes contact with religious or spiritual community — attendance, visits, ceremonies, funerals, elders, clergy, spiritual counsellors — the program supports access, including transportation and staffing wherever feasible. Contact preferences are recorded and reviewed at plan reviews.
Ceremony	Ceremonial practice, including Indigenous ceremony, is supported. Where ceremony requires specific conditions (fasting, smoke, plant medicines, time apart), the PC works with the person and the ceremony leader to make it possible in the home, involving the PM as needed. Ceremonial items and regalia are handled per the person's direction — often with specific protocols the person or their community will describe.
Interfaith households	In shared homes, the practice of one person is not curtailed to avoid inconvenience to another; where there is genuine tension, the PC facilitates a house conversation that respects all involved.

10. INDIVIDUAL CULTURAL CARE PLANNING

Every person in care has an Individual Cultural Care Plan (Appendix A), completed by the PC with the person at admission, updated at every change, and reviewed with the person at least annually. The plan records — only to the extent the person wishes to share — the person's culture, community, language(s), religion or spirituality, holidays and days of significance, foods and diet interacting with ECC-NIP §7, dress and grooming preferences, gendered care preferences, mourning and celebration practices, contacts and communities the person wants involved in their life, and anything else the person names.

- The plan is built with the person, at their pace. A blank plan is better than an assumed one. It is filled in over time as the person shares.

- Where the person is a member of an Indigenous Nation or governing body, Appendix B — the Indigenous Cultural Plan Supplement — is completed as well, in relationship with the person and, as the person authorizes, their community.
- Where the person is 2SLGBTQIA+, or where gender-affirming supports are relevant, Appendix C — the Gender-Affirming Care Plan — is completed alongside Appendix A. Appendices A and C are two lenses on the same person and reference each other; they are not filed against each other.
- The plans are living documents. Staff read them at orientation to the person and at every change; the PC verifies they remain current at every quarterly review and at every admission or major change to health status.

PART 3 — Indigenous Children, Youth and Adults

11. INDIGENOUS LEGAL FRAMEWORK AND RIGHTS

- Indigenous peoples in Canada hold rights recognized in the Canadian constitution, in UNDRIP as adopted through DRIPA in BC, and in the federal An Act respecting First Nations, Inuit and Métis children, youth and families. These rights include self-determination in matters concerning their children, families, cultures, and lands.
- For a child or youth in care (M): the CFCSA and related reforms centre cultural continuity and connection to family, community, and Nation. Indigenous governing bodies have increasing legal roles in decisions about their children. The program's role is to make cultural continuity real in the child's daily life and to receive and act on direction from the child's community wherever provided.
- For an adult (C): the adult is the decision-maker (ECC-HC Part 4). The program supports the adult's chosen relationships with their Nation, family, and community; supports self-determination in cultural and spiritual practice; and does not funnel Indigenous adults' decisions through a family or funder pathway that would not be applied to non-Indigenous adults.
- Where legal frameworks appear to conflict — for example, where a Nation's law comes into force in respect of a person in care — the PM works with the funder, the Nation, and the ED to align the program's response.

12. CONNECTION TO COMMUNITY, FAMILY AND NATION (M)

Requirement	Standard
At admission and continuously	The PC identifies (as the child or youth and their MCFD social worker share) the child's Nation, band or community, family, and community connections; the location of home territory; language(s); ceremonial or spiritual practices; and specific individuals — grandparents, aunts and uncles, elders, siblings — who are important to the child. Appendix B is completed and updated as information becomes available.
Contact plans	Contact with family, extended family, community, and Nation is a right of the child, subject to the direction of the social worker and any court order. The program schedules and supports contact — phone, video, visits, travel home — as an ordinary part of the plan, not an occasional event; where distance or funding is an

Requirement	Standard
	obstacle the PC escalates to the PM to problem-solve rather than accept loss of contact.
Cultural practices and ceremony	Cultural and ceremonial practices important to the child are supported: attendance at ceremonies, potlatches, powwows, feasts; regalia care; teachings from elders; land-based activities; language learning. Requirements the child or their community identify (fasting, seclusion, specific foods, absence from school for ceremony) are arranged for by the program with the social worker as authorization for schooling and any other systems requires.
Care planning done with the community where the child agrees	Where the child, the social worker, and the child's community agree, care planning meetings include the community; where the community has designated a person or role for such matters (a delegated worker, an elder, a specific auntie), that person is invited and their input carries. This is standard practice — not an unusual step.
Cultural continuity as a health outcome	Cultural continuity is protective — cultural disconnection increases risk. Staff document cultural connection observations (participation, comfort, requests, feedback) alongside other wellness observations. Loss of cultural connection is treated as a concern requiring response, not a normal consequence of placement.

13. INDIGENOUS CULTURAL PLANS AND PRACTICE (M)

Appendix B — the Indigenous Cultural Plan Supplement — sits alongside Appendix A for every Indigenous child or youth in care. It is completed with the child, and — with the child's agreement — with family and community. The plan records: identity (Nation, band, community; Métis or Inuit affiliation; urban Indigenous identity where the person names it); language(s); named contacts and their roles in the child's life; ceremonial and spiritual practices; important places and territories; foods, medicines, and cultural items; ceremonies and events on the calendar; teachings the child is receiving or wants to receive; the child's own goals for cultural connection; and the social worker's direction on cultural planning.

- The plan is co-authored, not extracted. Where the child does not want to share something, that is a legitimate answer.
- Where the child does not know parts of their history — a common consequence of removal — the program supports respectful reconnection with the social worker and community, without pressure and at the child's pace.
- The PC reviews Appendix B every quarter with the child; the PM reviews annually with the social worker.
- Staff-directed cultural activities based on generic assumptions ('let's do a smudge because they're Indigenous') are not cultural practice — they can be intrusive or harmful. Practices happen at the person's direction and with their community's guidance, or through relationships the program builds and holds for that purpose.

14. INDIGENOUS ADULTS (C) — CULTURAL SAFETY AND SELF-DETERMINATION

- For Indigenous adults, cultural safety and self-determination are exercised by the adult. Appendix A / B are built with the adult; contact with community is at the adult's choosing; ceremonial and spiritual practice, language, and connection to territory are supported at the adult's initiative and pace.
- Where CLBC planning processes touch cultural questions, the adult's decisions and priorities lead; the program supports; CLBC coordinates. This manual is clear that CLBC is not the cultural decision-maker for the adult.
- Where the adult wishes involvement of their Nation or Indigenous governing body in planning, health care, or advocacy, the program supports that connection; where the adult prefers not to be involved with the Nation, that is equally respected. Neither is more culturally safe — the adult's choice is culturally safe.
- The same supports as in Part 3 for children apply where the adult wants them: language reclamation, land access, ceremony, connection with family and community.

PART 4 — Gender-Affirming Care

15. GENDER-AFFIRMING CARE — PROGRAM STANDARD

Gender-affirming care is the ordinary work of every shift. The evidence — from BC clinical guidance (Trans Care BC), from Canadian and international standards, and from the lived experience of the people we serve — is that affirmation of a person's gender identity and expression measurably reduces distress, self-harm, and suicide risk in 2SLGBTQIA+ children, youth, and adults. Non-affirming environments do the opposite. This is a safety question, not a debate.

Standard	Program Practice
Affirm on the person's terms	The person's own account of their gender identity and expression is authoritative. Staff do not require a diagnosis, a clinician's letter, family agreement, a legal change, or any other authorization to affirm the person in daily life.
No conditions, no delay	Names, pronouns, clothing, hygiene supplies, and access to safe spaces are provided from the first day — not held pending behaviour, family, or other conditions. There is no probation period on a person's identity.
No commentary, no policing	Staff do not comment on a person's body, gender presentation, voice, or how 'convincing' they look; do not police clothing choices by gender; and do not correct other staff or residents publicly in ways that draw attention to a person's identity. Corrections happen — see Section 16 — but privately and without shame.
Privacy is a core condition of safety	A person's gender identity, sexual orientation, trans status, HIV status, immigration status, and related information are among the most sensitive personal information the program holds. Disclosure is on the person's terms only (Section 22). Outing is a prohibited

Standard	Program Practice
	practice.
The person is not the educator	Staff bring themselves up to speed on gender-affirming care through the training required by this manual and available BC resources — not by requiring the person in care to explain themselves, teach the basics, or serve as a case study.
Consent frameworks apply — the person leads	For everyday practice: the person's own preferences govern; no external consent is required to use a name and pronoun. For health care under gender-affirming care: children and youth (M) may consent under the Infants Act where the provider is satisfied of capability (ECC-HC §12); adults (C) consent for themselves (ECC-HC Part 4). Section 19 details the program's role.

16. NAMES, PRONOUNS, DOCUMENTATION AND PRIVACY

Requirement	Standard
Name and pronouns as the person states them	From the first shift and every shift thereafter, the person is addressed by the name and pronouns they use — including where these differ from legal documents, family use, or historical records. This is not a preference to be balanced against other views; it is the standard.
When the person's chosen name and legal name differ	The person's chosen name is used in every daily context: staff speech, house lists, meal plans, program schedules, most internal records where identity is not legally required. Legal documents (medication prescriptions, MSP, court documents, official reports) use the legal name because the systems require it — the person is informed of this, and staff take care to prevent legal-name exposure in daily settings (for example, not calling the legal name aloud at pharmacy pickup where the chosen name is on the phone-in note; discussing options with the pharmacy for alignment).
Changing records	Where the person chooses to update their legal name and gender marker, the program supports the process (BC Vital Statistics, MSP, ICBC) at the person's pace, without pressure. Changed records propagate through program systems the same day the change is documented.
Correcting mistakes	Staff who use the wrong name or pronoun correct themselves briefly, without drawn-out apology or asking for reassurance from the person, and move on. Repeated errors by the same staff member trigger PC coaching (Section 25).

Requirement	Standard
Correcting other people	Other residents, family visitors, contractors, or community members who use wrong names or pronouns are corrected by staff in the moment where the person in care has not asked otherwise — 'Actually, her name is Nadia and she uses she/her' — and briefed in advance where a visit is upcoming and the person consents to that briefing.
No outing	A person's gender identity, sexual orientation, trans status, or previous name is not disclosed to family, school, work, other residents, or any third party without the person's specific consent for that disclosure. Section 22 governs. This includes not disclosing to funder staff beyond the minimum operationally required, and never as small talk.
Deadnames	The person's previous name is not used in daily life; where it must appear in a legal record, it appears as it must and no more. Historical documents in the person's file that use the previous name are not deleted (record integrity) but new documents use the current name; a cover sheet notes the name change so old and new records can be linked for care continuity.
Documents and photos	Photos, ID cards, transit cards, and other daily-use documents are updated to the current name and photograph as soon as feasible; the PC supports issuance.

17. EVERYDAY PRACTICES — CLOTHING, HYGIENE, PRESENTATION

- The person chooses their clothing, hair, grooming, and self-presentation. The program's role is to make choices available and to respect them.
- Clothing budgets and shopping include the person's choices without gendered assumptions. Where a person's presentation differs from what the funder or family have historically purchased, the PC supports appropriate acquisition through the person's clothing budget and any available community resources.
- Hygiene and grooming supplies (hair care, skin care, shaving, chest binders, gaffs, tucking underwear, breast forms, packing prosthetics, menstrual products regardless of gender) are provided as the person requires. The PC arranges supply confidentially and consistently, working with pharmacies and community organizations that serve trans and non-binary people.
- Chest binders: safe use guidance is offered where the person wishes (fit, wear time, breaks, alternatives) drawing on Trans Care BC resources. The program does not require binder use, does not restrict it, and does not gatekeep supply. Any health concern raised is addressed as a health question under ECC-HC and ECC-IIME.
- Hairstyles, makeup, jewellery, nail care, and other presentation choices are supported. Where a person is a young child (M), staff support the child's stated preferences and the direction of the consent authority (ECC-HC §11) — with the strong presumption in favour of the child's own preferences on daily presentation.

- Where an activity or setting has a dress code (school, work, ceremony, religious event) that raises questions for the person, the PC helps the person navigate — informed by the person's own priorities — and does not impose an outside expectation as a program requirement.

18. SAFE SPACES — ROOMING, WASHROOMS, BATHING, ROOMMATE

Setting	Standard
Room	Placement is by the person's own gender identity, not by sex assigned at birth. In shared homes, the PC works with the person to arrange rooming that is safe for them; a single room is preferred where feasible for trans and non-binary people at any age. A person's room is private space in which they present as they wish; no staff enter without knocking and being welcomed except in emergency.
Washroom and bathing	The person uses the washroom that matches their gender identity where the program has multiple. Where a single-user washroom is available it is offered but never required or assigned. Bathing supports (where staff assistance is needed) are provided by staff of the gender the person prefers where feasible; the preference is recorded on Appendix C and the schedule is built to it. Preferences may vary among family showers, water sports, medical settings — Appendix C records each context.
Roommate and household composition	Where new placements or roommate changes may affect a 2SLGBTQIA+ person's safety or comfort, the PC consults the person before the change and takes their view seriously in the decision. A safety concern raised is not overridden by administrative convenience.
Personal search / body assistance / clothing changes	Staff of the gender the person prefers provide assistance where possible; the person's dignity and privacy come first; searches are avoided as a matter of practice (ECC-BSP) and where they occur they follow the person's plan; medical exams are handled per ECC-HC with the person's specifications about chaperones and provider gender.
Sports, activities, groups	Participation is by the person's own gender identity. Where a school, sports league, or community activity imposes different rules, the PC works with the person to problem-solve and, with the person's consent, advocates. The program does not enforce external rules by default.

19. CONSENT AND ACCESS TO GENDER-AFFIRMING HEALTH CARE

Gender-affirming health care — from social affirmation supports through medical care such as hormone therapy and surgical care — is provided by health care providers. The program's role is access: identifying

providers, arranging appointments, supporting attendance, coordinating logistics, and — under ECC-HC — supporting consent processes led by the person and the provider.

Stream	Standard
Children and youth (M)	A minor may consent to their own health care, including gender-affirming health care, where the treating health care provider is satisfied of capability under Infants Act s.17 (ECC-HC §12). The program does not gatekeep, delay, or route the consent process through the social worker as though the social worker's consent were legally required where s.17 applies — that determination is the provider's. Where s.17 does not apply (the provider is not satisfied of capability for the specific decision, or the youth prefers involving their consent authority), the process follows ECC-HC §11 with the social worker per legal status. In all cases the program supports the youth's participation, attends appointments as the youth wishes, and helps arrange follow-up.
Adults (C)	Adults are presumed capable (HCCCFAA s.3) and consent for themselves. The program supports access on the adult's terms; substitute decision-making applies only where a provider determines incapability for a specific decision (ECC-HC Part 4) — which is not a general status and never a program-issued label.
Choosing providers	The PC maintains contact information for Trans Care BC (the provincial resource), gender-affirming primary care providers, endocrinology and specialist services, and community organizations. Where the person has an existing provider, that continuity is maintained on placement change wherever possible.
Confidentiality of care	Attendance at gender-affirming appointments, the fact of receiving care, and the nature of any treatment are among the most private information the program holds. Section 22 governs. Family and funder disclosures are only on the person's consent for that disclosure.
Medications and monitoring	Prescriptions for gender-affirming medications are handled through the Medication Administration Manual with no additional restrictions or scrutiny relative to any other medication (ECC-MED §22 psychotropic-like monitoring is applied only where clinically directed). Missed doses and refills are treated with the same urgency as other important medications.
Practice restrictions on staff	Staff do not offer clinical advice on gender-affirming health care — dosing, timing, when to seek care, whether care is 'necessary' — beyond following the person's prescriber's directions. Staff do not tell a

Stream	Standard
	person that they should or should not pursue any specific gender-affirming health care. This is not equivocation; it is scope of practice.

20. FAMILY, COMMUNITY AND PEER CONTACT

- The person chooses which family and community are informed about, involved in, or excluded from their gender identity and gender-affirming care. Family involvement is important where the person wants it and safe where the person judges it safe.
- Where a family member responds to a person's identity in ways that harm the person (rejection, insistence on deadname, threats, refusal of contact conditional on 'stopping'), the program supports the person, follows the person's direction about contact, and — for youth (M) — informs the social worker so that appropriate protection and planning responses can occur.
- The program supports contact with peer and community supports — 2SLGBTQIA+ youth groups, Indigenous 2S groups, adult drop-ins, online communities the person uses — as ordinary and normal. These are health supports, not extras.
- Where a person is dating or in a relationship, the program treats those relationships with the same respect, privacy, and support as any other person's relationships, subject to the ordinary safety and age-appropriate frameworks in ECC-BSP and ECC-HC.

21. WHEN A PERSON COMES OUT AT THE PROGRAM

Step	Standard
1. Listen and thank	When a person tells a staff member about their gender identity, sexual orientation, or related information, the staff member listens, thanks the person for telling them, and asks what the person would like to happen next — including who they want the information shared with, and who they specifically do not want it shared with.
2. No inquiry, no test	Staff do not ask when the person 'figured it out', whether they are sure, whether they have told family, or any other question that treats the disclosure as something to be verified. Openness to answering the person's questions is different from interrogating them.
3. Immediate practice	The name and pronouns the person tells us are used from that moment. Wardrobe, hygiene, and other daily needs identified are supplied through the ordinary process (Section 17). The person is not asked to wait for a plan meeting before being addressed as they wish.
4. Plan and privacy	The PC updates Appendix C (Gender-Affirming Care Plan) with the person as soon as the person is ready; the plan records specifically who the person wants informed, when, and by whom — often the person

Step	Standard
	themselves. Nothing is shared beyond staff on a need-to-know basis without the person's direction (Section 22).
5. Support	The PC offers connection with peer supports and, if the person wishes, gender-affirming clinical services. Offers are made, not imposed; a person's choice not to pursue any specific supports is respected.
6. Ongoing check-in	Coming out is not a single event. The PC checks in periodically about how the plan is working, changes the person wants, and any incidents that need attention. Appendix C is updated at every change.

PART 5 — Records, Complaints and Escalation

22. RECORDS AND CONFIDENTIALITY

Requirement	Standard
What is recorded	Cultural, religious, spiritual, and identity information is recorded only to the extent needed to support the person's care and only where the person has agreed. The plans in Appendices A–C are the primary places this information lives; it is not scattered through daily logs unnecessarily.
Access — need to know	Access to identity-sensitive information is on a need-to-know basis: staff providing direct care to the person; the PC and PM. Funder staff receive information as required by contract and only to the minimum necessary. Reception, contractors, and other non-care staff do not receive it as a matter of course.
Correspondence and calendars	Program calendars, house lists, dietary boards, and other visible documents use the person's current name and pronouns. Cultural or identity information is not put on visible documents unless the person has agreed.
Disclosures outside the program	Disclosure to family, schools, employers, community programs, or health providers about a person's culture, faith, gender identity, sexual orientation, or related information happens only with the person's specific consent for that disclosure (children (M) with attention to Infants Act mature-minor considerations and social worker involvement per care plan; adults (C) with the adult's own consent — never through a substitute unless legally required for a specific incapable decision under HCCCFAA).
Retention and destruction	Records follow the agency retention schedule;

Requirement	Standard
	identity-sensitive information is not retained beyond the retention period; where a person has revoked consent to a specific record's continued use, the PC works with the ED and, where applicable, the funder to align retention with the person's rights under privacy law.
Records at transfer / discharge	Cultural and identity information travels with the person only to the receiving service the person authorizes. Historic legal names are handled as in Section 16.

23. CONCERNS, COMPLAINTS AND REPORTING HARM

Route	Standard
Everyday concerns	The person, family, community, or staff raise concerns to any staff member; concerns are recorded on Appendix E (Concerns and Complaints Log) and passed to the PC same shift; the PC responds within 5 business days with the person and follows up in writing where the person prefers.
Serious concerns and complaints	Where a concern is about staff conduct, a serious pattern, or a specific harmful incident, the PM leads response: acknowledgement to the complainant within 2 business days; investigation; response and any corrective action within 20 business days; documentation in the individual record and in the corrective action log.
Identity-based harm incidents	Racism, transphobia, homophobia, ableism, or other identity-based harm incidents are always reported and always responded to (Section 7). Where the incident meets funder reportable criteria (harm to the person; involvement of staff; a pattern), the ECC-IIME §17 notification matrix applies: MCFD social worker and licensing where applicable (M) or CLBC per contract (C). Where the incident is a criminal act (assault; hate crime), police are contacted with the person's involvement in the decision to the extent possible.
Complainant's right to advance without the program	Complainants are told, in language they understand, of external routes: the funder (MCFD complaints; CLBC complaints); the Representative for Children and Youth (M) — a statutory advocate for children in care; the BC Ombudsperson; the BC Human Rights Tribunal; the BC Office of the Information and Privacy Commissioner (privacy); the applicable regulatory body for professional conduct. The program supports complainants who wish to use these routes and does not retaliate.

Route	Standard
No retaliation	Nobody who raises a concern faces retaliation: not in care, not in employment, not in family contact facilitated by the program. Retaliation is grounds for HR action.
Debriefs and learning	After significant incidents, the PC offers a debrief to the person and family (as they wish), to affected staff, and — for identity-based harm from outside the program — to the roommate/household group as appropriate. Every complaint feeds the annual review (Section 25).

PART 6 — Quality Assurance

24. QA INDICATORS — CULTURAL SAFETY AND GENDER-AFFIRMING CARE

Indicator	Target	Frequency
Appendix A (Individual Cultural Care Plan) current for every person; updated at admission and annually	100%	Monthly PC audit
Appendix B (Indigenous Cultural Plan Supplement) current for every Indigenous person	100%	Monthly PC audit
Appendix C (Gender-Affirming Care Plan) current for every person for whom it applies (and offered to every person)	100%	Monthly PC audit
Interpretation events (Appendix D) — every significant conversation for a person whose language is not shared by staff	100%; no significant conversations conducted through family, roommates, or children	Per event; monthly audit
Indigenous Cultural Safety training — all staff (San'yas or equivalent BC-recognized ICS training)	100% within 6 months of hire; annual refresh where offered	Monthly staff currency check
Gender-affirming care training — all staff before unsupervised duties (SHSS G.3.2(b) requirement — M; adopted for all streams)	100%	Monthly staff currency check
Concerns and complaints (Appendix E) responded to within stated timelines	100%	Monthly PC audit; PM review
Identity-based harm incidents — reported and investigated with corrective action	100%	Per event; PM personal confirmation of any funder notification (with ECC-IIME §17)

Indicator	Target	Frequency
Person-in-care feedback: has care been culturally safe and affirming?	Actively solicited at admission, quarterly, at discharge, and any time the person wishes; feedback drives change	Quarterly and per event
Programs maintain relationships with named local Indigenous and 2SLGBTQIA+ organizations	Named contacts on file; contact tested at least twice a year	Quarterly PM check-in

25. ANNUAL REVIEW AND TRAINING

- This manual is reviewed annually by July 1 by the PM and approved by the ED, and immediately upon any funder directive, licensing change, incident revealing a policy gap, or feedback from persons in care, family, community, or partner organizations.
- Every staff member completes cultural safety training and gender-affirming care training before unsupervised duties. The current program standard: (a) a BC-recognized Indigenous Cultural Safety training (San'yas Indigenous Cultural Safety, or provincial equivalent) within 6 months of hire and refreshed as the training program directs; (b) gender-affirming care training grounded in Trans Care BC and consistent with SHSS G.3.2(b) before duties, refreshed annually.
- Orientation to this manual covers each Part; the Appendices are worked through with example scenarios; the response to identity-based harm incidents (Section 23) is walked through explicitly, including the notification chain and the person-supporting steps.
- The PM sustains program-level relationships with Indigenous organizations and 2SLGBTQIA+ community supports and reviews the resource roster annually.
- Feedback and complaints feed the annual review; each generates at least one corrective action with a named owner and target date, tracked in the Corrective Action Log — mirroring ECC-MED §31.

PART 7 — Appendices

APPENDIX A — INDIVIDUAL CULTURAL CARE PLAN (PER PERSON)

Completed by the PC with the person at admission and updated at every change and at least annually. The person shares what they wish to share; blanks are legitimate. Where the person is Indigenous, Appendix B is completed alongside; where gender-affirming supports are relevant, Appendix C is completed alongside. All three plans reference each other and are read together.

Person's Legal Name / Preferred Name	
Date of Birth	
Stream (M / C)	
Culture(s), community, ancestry as the person names them	
Language(s) — for daily life • for health care • for confidential conversations	
Religion / spirituality / practice (or none) — days of significance, prayer, dietary	

observance (cross-reference ECC-NIP §7), dress, ceremonial items, sacred spaces in the person's room	
Community and family — named contacts the person wants in their life; nature of contact; consent for staff to reach out	
Cultural activities and practices important to the person; how the program supports them	
Gendered care preferences — bathing, personal care, medical (with cross- reference to Appendix C)	
Foods and diet — cultural, religious, personal preferences (with cross-reference to ECC-NIP Appendix B)	
Mourning, celebration, life-event practices important to the person	
Anything else the person wishes to share; anything the person specifically does not want shared and with whom	
Local resources and named community contacts on file	
PC Signature / Date	
Person's Signature / Date (or reason not signed at this time)	
PM Approval — Signature / Date	
Review Dates / Initials (at least annually and at every change)	

APPENDIX B — INDIGENOUS CULTURAL PLAN SUPPLEMENT (PER PERSON)

** Completed with every Indigenous person in care, in relationship with the person and — as the person authorizes — their family, community, and delegated Indigenous authority. Co-authored, not extracted. Where the person does not know parts of their history, the plan supports respectful reconnection at the person's pace. **

Person's Legal Name / Preferred Name	
Date of Birth	
Identity as the person names it (Nation, band, community; Métis or Inuit affiliation; urban Indigenous identity; other)	
Home territory / land / places of	

significance	
Language(s) — status (fluent / learning / reclaiming / not currently accessed) and supports the person wants	
Named contacts — family, elders, aunties/uncles, ceremonial and spiritual supporters, community and Nation contacts; the role each plays in the person's life; consent for staff to reach out	
Ceremonial and spiritual practice — ceremonies, feasts, potlatches, powwows, land-based activities; regalia and cultural items and their care	
Teachings currently being received or the person wants to receive; teachers and mentors	
Direction from the Nation / Indigenous governing body about the person's care (where provided)	
Social worker direction on cultural planning (M) — attach current instructions	
Person's own cultural goals for this year	
Program-level relationships supporting this plan — Friendship Centre, Nation contact, elders, cultural workers	
PC Signature / Date	
Person's Signature / Date (or reason not signed at this time)	
Community authorization / signatures where applicable and authorized by the person	
PM Approval — Signature / Date	
Quarterly / Annual Review Dates / Initials	

APPENDIX C — GENDER-AFFIRMING CARE PLAN (PER PERSON)

Offered to every person; completed with any person for whom gender-affirming supports are relevant, at the person's pace and on their terms. Nothing on this plan is disclosed to anyone outside the on-shift care team without the person's specific direction. Deadnaming and outing are prohibited practices (Sections 16 and 22).

Person's Legal Name (as it appears on legal documents) — used only where legally	
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required	
Chosen Name — used in every daily context	
Pronouns — used every time	
Gender identity as the person names it (only if the person wishes to record it)	
Who knows / who is to be told / who is NOT to be told (family, roommates, school/work, community, funder staff)	
How disclosures happen — by the person, by the PC with the person's specific consent, or not at all — for each context above	
Everyday supplies and supports needed (hair, skin, shaving, chest binder, gaff/tucking, breast forms, packing, menstrual products regardless of gender, other) — how supplied	
Room, washroom, bathing, and personal care preferences — including gender of staff providing assistance in each context	
Clothing budget notes; documents to update (BC Vital Statistics, MSP, ICBC, school, employer) with target dates and the person's chosen pace	
Gender-affirming health care — current care, providers, appointments (kept confidential per Section 22); consent framework noted (Infants Act s.17 (M) / HCCCFAA (C))	
Peer and community supports the person is connected to or would like to be connected to	
Safety concerns — situations, people, places where the person feels unsafe; the plan around each	
What is going well; what the person wants to change	
PC Signature / Date	

Person's Signature / Date (or reason not signed at this time)	
PM Approval — Signature / Date	
Review Dates / Initials	

APPENDIX D — INTERPRETATION AND LANGUAGE ACCESS RECORD

One log per person. Every significant conversation for a person whose preferred language is not shared by staff is entered here. Family, roommates, and children are not used as interpreters for significant conversations.

Person's Legal Name / Chosen Name				
Preferred Language(s) for daily / health / confidential contexts (from Appendix A)				
Date	Purpose (care plan / health appointment / complaint / other)	Interpreter — service, name, professional?	Mode (in person / phone / video)	Follow-up needed

APPENDIX E — CONCERNS AND COMPLAINTS LOG

Every concern about cultural safety, gender-affirming care, or identity-based harm is entered here. Serious complaints route to the PM per Section 23; identity-based harm incidents also engage the ECC-IIME §17 notification matrix where criteria are met.

Date received	Raised by (person in care / family / community / staff / other)	Concern (in the complainant's words as far as possible)	Response / actions taken (with dates)	Timeline met? (5 bd concern / 20 bd complaint)	External route offered / used?	Outcome and closure — signed by PC/PM

APPENDIX F — STAFF ACKNOWLEDGEMENT

All staff, volunteers, students, and contractors must read and sign this acknowledgement before unsupervised duties and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Cultural Safety & Gender-Affirming Care Manual (ECC-CSGA-2026-001), Version 1.0, effective July 1, 2026.	Initials
I understand that each person is the authority on their own culture, community, religion, language, name, pronouns, and gender identity, and that I use what they tell me from the first shift without requiring justification.	
I understand that Indigenous Cultural Safety is a baseline standard and specifically shapes how I support Indigenous people in our homes; I will complete the required BC-recognized ICS training within 6 months of hire and follow the direction of the person's community where provided.	
I understand that gender-affirming care training (SHSS G.3.2(b)) is required before unsupervised duties and that I use each person's name and pronouns every time, provide access to the supplies and safe spaces they need (Sections 17–18), and never delay or condition affirmation.	
I understand that outing — disclosing a person's gender identity, sexual orientation, previous name, or related information — is a prohibited practice, and that I share identity-sensitive information only on a need-to-know basis and with the person's direction (Section 22).	
I understand that I will interrupt and report any incident of racism, transphobia, homophobia, ableism, or other identity-based harm — including from staff, roommates, family, contractors, or community — the same shift, on Appendix E, and that I will not retaliate against anyone who raises a concern.	
I understand that consent for gender-affirming health care and for other health care follows ECC-HC: children and youth (M) may consent under the Infants Act where the provider is satisfied of capability; adults (C) consent for themselves under the HCCCFAA; substitute frameworks apply only where the provider determines incapability for a specific decision.	
I understand that staff do not provide clinical advice on gender-affirming health care beyond following the prescriber's directions, and do not tell a person that they should or should not pursue any specific care.	

I acknowledge that I have received, read, and understood the Evergreen Community Care Cultural Safety & Gender-Affirming Care Manual (ECC-CSGA-2026-001), Version 1.0, effective July 1, 2026.		Initials
I understand that professional interpreters are used for significant conversations, and that family, roommates, and children are not used as interpreters.		
Staff / Volunteer / Student / Contractor Name (print)		
Position / Role		
Program		
Signature		
Date		
ICS training completed — course, date		
Gender-affirming care training completed — date		
PC Signature (training current, on file) / Date		

VERSION HISTORY

Version	Date	Summary
1.0	July 1, 2026	Initial Cultural Safety & Gender-Affirming Care Manual: cultural safety and humility as the program standard; Indigenous Cultural Safety grounded in DRIPA/UNDRIP, In Plain Sight (2020), and the federal Act respecting First Nations, Inuit and Métis children, youth and families; anti-racism response; language and interpretation access with the family/roommate/children prohibition on significant conversations; religious and spiritual practice; Individual Cultural Care Plan; dedicated Part on Indigenous children, youth (M) and adults (C) with cultural continuity as an outcome and community direction as authoritative; full Part on gender-affirming care — names, pronouns, records, everyday practice, safe spaces, family/community/peer contact, and consent and access to gender-affirming health care under Infants Act s.17 (M) and HCCCFAA (C); prohibited practices — outing, deadnaming, delay or conditions on affirmation, staff-directed cultural activities based on generic assumptions; records and confidentiality; concerns and complaints with external routes (Representative for Children and Youth, Ombudsperson, BC Human Rights Tribunal); QA indicators; training requirements — BC-recognized ICS training within 6 months of hire and gender-affirming care training (SHSS G.3.2(b)) before unsupervised duties; 6 appendices.

Executive Director	Signature: _____ Date: _____
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